

Threadworm: Pharmacy Protocol for P Medicine Sale

Version 001, issued 8 September 2026. Mebendazole 100mg for threadworm, sold as a Pharmacy (P) medicine. This document replaces the withdrawn Threadworms PGD, which is retired.

Why this is a protocol and not a PGD

A Patient Group Direction is the legal mechanism for supplying a Prescription Only Medicine without a prescription. It is only needed for a POM.

- **Mebendazole 100mg for threadworm in anyone aged 2 years and over is a PHARMACY (P) medicine. Ovex Suspension and Ovex tablets are both P.**
- **A P medicine is sold under the supervision of a pharmacist. It does not need, and should not have, a PGD.**
- **The Threadworms PGD is therefore retired rather than reissued. This protocol replaces it and sets out the checks, the correct dose, and the record to keep.**
- **This is the same conclusion reached on the ibuprofen arm of the dental PGD, which was removed for the same reason and handled as a P sale with a consultation record.**

THE DOSE. Read this before anything else.

- **MEBENDAZOLE ORAL SUSPENSION IS 100mg IN 5mL. A 100mg dose is 5 mL. ONE 5 mL SPOONFUL.**
- **The withdrawn PGD stated the suspension as 5mg/mL and gave the 100mg dose as 20 mL, supplying 40 mL. Measured against the actual product, 20 mL is 400mg, four times the intended dose, and the 40 mL supplied was 800mg. That error is why the PGD was withdrawn on 7 September 2026.**
- **Tablets: one 100mg tablet is one dose. There is no calculation to do.**
- **The dose is the same for everyone aged 2 and over. It is not weight-based. Do not scale it for a small child or a large adult.**

Mebendazole 100mg for threadworm

Protocol for the sale of mebendazole 100mg tablets or 100mg/5mL oral suspension as a Pharmacy medicine, for the treatment of threadworm (*Enterobius vermicularis*) in adults and children aged 2 years and over.

Healthcare professionals covered, and training

- The sale must be made by, or under the supervision of, a pharmacist.
- Anyone conducting the consultation must be able to recognise the presentation of threadworm and to distinguish it from other causes of perianal itch.
- Anyone conducting the consultation must know that mebendazole is CONTRAINDICATED IN PREGNANCY, and must ask.
- Anyone conducting the consultation must know that metronidazole must not be taken concomitantly with mebendazole.
- Anyone conducting the consultation must be able to explain the hygiene measures, which do more to prevent reinfection than the medicine does.
- Refer to the pharmacist any request outside this protocol, including children under 2, pregnancy, and repeated or persistent infection.

The PGD

When to sell	• Aged 2 years and over. • Confirmed or clinically suspected threadworm: perianal itch, worse at night, sometimes with visible thread-like worms in the stool or around the anus, disturbed sleep, or irritability in a child. • Consent from a parent or guardian where the patient is a child. • Able to take an oral medicine. • No reason to refer, below.
When to refer instead of selling	Do not sell. Refer to the GP where any of the following applies. • PREGNANT, OR PREGNANCY POSSIBLE. Mebendazole is CONTRAINDICATED in pregnancy by the SPC, in every trimester. Hygiene measures alone, and GP advice. • Under 2 years of age. Mebendazole is not recommended below 2, must not be used below 1, and between 1 and 2 only if the benefit justifies the risk, which is a prescriber decision. Hygiene measures alone; refer. • Taking METRONIDAZOLE, or has taken it recently. Concomitant use with mebendazole should be avoided: it has been associated with an outbreak of Stevens-Johnson syndrome and toxic epidermal necrolysis. • Breastfeeding. A small amount passes into milk; the SPC advises caution. Refer for advice rather than selling over the counter. • Known hypersensitivity to mebendazole or to any excipient. • Taking cimetidine, which inhibits mebendazole metabolism and raises plasma levels. • Liver disease, or a history of abnormal liver function. • Rare hereditary fructose intolerance, glucose-galactose malabsorption or sucrase-isomaltase insufficiency. The suspension contains 500mg sucrose per 5 mL. • Symptoms persisting after two correctly taken courses, or a third episode in a short period. Something else may be going on. • Weight loss, abdominal pain, blood in the stool, or any symptom not explained by threadworm. • Recent foreign travel with gastrointestinal symptoms, where another parasite may be involved.
What to sell	ADULTS AND CHILDREN 2 YEARS AND OVER: • One 100mg tablet, chewed or swallowed. OR one 5 mL spoonful of 100mg/5mL suspension. Shake the suspension before use. • A second dose after 2 weeks is highly recommended where

	reinfection is suspected, which in a household with children is usual. TREAT THE WHOLE HOUSEHOLD AT THE SAME TIME. Everyone aged 2 and over, whether or not they have symptoms. This is the single thing that most affects whether the infection comes back. - Count the household and sell enough for everyone, plus the repeat dose where indicated. Anyone in the household who is pregnant, or under 2, is not treated. Hygiene measures for them, and GP advice where needed.
Hygiene measures. These matter more than the tablet	Give these every time, in writing. Reinfection is by swallowing eggs from hands, bedding and surfaces, and the medicine does not kill eggs. - Wash hands and scrub under the nails first thing in the morning, after using the toilet, and before every meal. - Keep fingernails short. Discourage nail biting and thumb sucking. - Shower or bath every morning, including washing around the bottom, to remove eggs laid overnight. - Change and wash underwear, nightclothes, bed linen and towels daily for the first few days, at a hot wash. - Do not shake bedding or clothing, which scatters eggs into the air. - Vacuum and damp-dust bedrooms and bathrooms, and clean toys and hard surfaces. - Wear close-fitting underwear at night, and avoid scratching. Keep this up for 2 weeks after the dose, which is how long eggs can survive in the house.
What to record	A P sale still needs a record. Keep it in the consultation record, not just on the till. - Name, and age or date of birth, of everyone being treated. That pregnancy was asked about, and the answer. That the metronidazole question was asked, and the answer. - Number of household members treated, and anyone excluded and why. - Product, form, strength and quantity sold. That the dose was given as ONE 5 mL spoonful or ONE tablet, and that hygiene advice was given. - Name of the person making the sale and the supervising pharmacist. - Any advice given where a sale was declined, and the referral made.

The medicine

Products	Mebendazole 100mg chewable tablets and mebendazole 100mg/5mL oral suspension. Ovex and Ovex Suspension (McNeil) are the common P-labelled brands; Vermox tablets and Vermox 100mg/5mL oral suspension (Janssen-Cilag) are POM and are not sold under this protocol.
Legal category	P (Pharmacy). Sale under the supervision of a pharmacist. No PGD is required or permitted for a P medicine.
Strength, stated plainly	Suspension: 100mg per 5 mL. Tablets: 100mg each. A single dose is 5 mL or one tablet.
Storage	No special storage conditions. Shake the suspension before use.
Interactions	Avoid concomitant metronidazole. Cimetidine inhibits mebendazole metabolism and raises plasma levels. Refer where either applies.
Adverse effects	Uncommon: abdominal pain, abdominal discomfort, diarrhoea, flatulence, rash, dizziness. Rare: nausea, vomiting, hepatitis and abnormal liver function tests, neutropenia, convulsions, hypersensitivity including anaphylaxis, and severe skin reactions including Stevens-Johnson syndrome and toxic epidermal

	necrolysis. Convulsions have been reported very rarely in children, including infants under one year.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk . A P medicine sale is reportable in exactly the same way as a prescription supply.
Disposal	Advise the customer to return any unused medicine to a pharmacy.

Information for the patient

Written information	Give the patient information leaflet supplied with the product, and the hygiene advice above in writing. The hygiene sheet is the part that gets kept.
Counselling	● One 5 mL spoonful, or one tablet. That is the whole dose. Shake the bottle first. ● Everyone in the house aged 2 and over should take it on the same day, even if they have no symptoms. ● Take a second dose in 2 weeks if you think it has come back, which is common. ● Threadworm is extremely common in children and is nothing to be embarrassed about. It is not a sign of a dirty home. ● The hygiene measures are what stop it coming back. The medicine kills the worms; it does not kill the eggs.
When to come back	● If the itching is no better a week after the second dose. ● If there is tummy pain, weight loss, or blood in the stool. ● If a rash, swelling of the face or lips, or any reaction develops. Stop the medicine. ● If anyone in the house is pregnant or under 2, come and talk to us about what they should do instead.

Appendix 1: Key references

- Summary of Product Characteristics for Ovex Suspension, PL 15513/0313, text revised 10 December 2025, and for Ovex tablets.
- Summary of Product Characteristics for Vermox 100mg tablets and Vermox 100mg/5ml oral suspension.
- NICE Clinical Knowledge Summary: Threadworm.
- British National Formulary and BNF for Children.
- Human Medicines Regulations 2012, for the legal categories of medicines.

Authorisation

Version	Protocol v001
Supersedes	Threadworms PGD, withdrawn 7 September 2026 and retired 8 September 2026
Valid from date	8 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 8 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 8 September 2026

Both authorising signatories reviewed this version together on 8 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	Protocol v001
Date	8 September 2026
Changes	• New pharmacy protocol replacing the retired Threadworms PGD. Mebendazole for threadworm at 2 years and over is a P medicine and a PGD is only required for a POM. • The dose is stated as ONE 5 mL spoonful of 100mg/5mL suspension, or one 100mg tablet. The withdrawn PGD stated the suspension as 5mg/mL and gave the dose as 20 mL, which against the real product is 400mg, four times intended, in a mostly paediatric service. • Pregnancy is a straight referral. The withdrawn PGD permitted supply in the second and third trimesters; the SPC contraindicates mebendazole in pregnancy outright. • Metronidazole added as a referral. The withdrawn PGD named cimetidine as the only interaction and did not mention metronidazole, which the SPC says to avoid with mebendazole following a Stevens-Johnson syndrome and toxic epidermal necrolysis outbreak. • Under 2 handled properly: not recommended below 2, not to be used below 1, and 1 to 2 only on a prescriber decision. • Whole-household treatment made an explicit instruction with a count, rather than a line of follow-up advice. • Hygiene measures expanded and moved to the front of the patient-facing section, on the basis that they prevent reinfection and the medicine does not. • A record requirement added for a P sale, covering the pregnancy and metronidazole questions and the dose actually advised.

Previous versions

Threadworms PGD, withdrawn 7 Sep 2026	Mebendazole 100mg tablets and oral suspension, ages 2 and over. Withdrawn after the suspension strength was found to be stated as 5mg/mL against an actual 100mg/5mL, making the stated dose four times intended. Retired rather than reissued on 8 September 2026.
--	---